Breast adjuvant treatment only · NCCP 00588h. These materials cover olaparib after treatment for high-risk early breast cancer. Other cancers use different eligibility, duration and follow-up; use the matching NCCP indication and your treating team's plan.

NCCP 00588 · VERSION 5B

Olaparib · adjuvant breast treatment

Take olaparib tablets morning and evening every day. There is no planned week off. Blood-review appointments are usually every four weeks during this one-year breast course.

Oral targeted therapy · PARP inhibitor · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Nausea and appetite

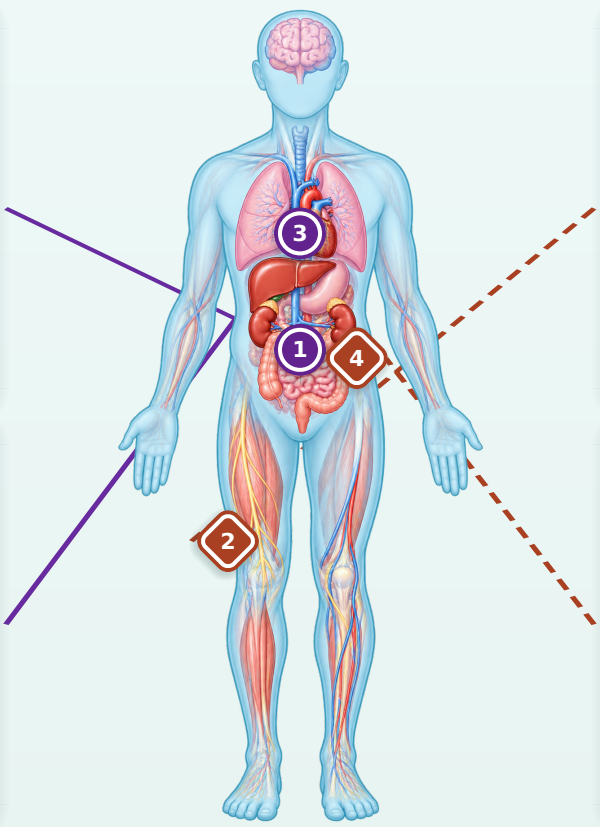
Olaparib

Ask for help early if nausea or vomiting makes it difficult to eat, drink or take tablets.

2 Blood counts and energy

Olaparib

Anaemia can cause marked tiredness or breathlessness; low white cells and platelets can cause infection or bruising.



3 New breathing symptoms

Olaparib

A new cough or worsening breathlessness needs prompt assessment; uncommon lung inflammation is important to recognise.

4 Liver warning signs

Olaparib

Report jaundice, dark urine or persistent upper abdominal discomfort; liver tests help investigate possible injury.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team urgently for fever or feeling acutely unwell, unusual bleeding, new or worsening breathlessness/cough, severe weakness, persistent vomiting or inability to drink. Chest pain, severe breathing difficulty or collapse require emergency help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

YOUR PRACTICAL TREATMENT PLAN

Know the next step.

Taking treatment

Take olaparib tablets morning and evening every day. There is no planned week off. Blood-review appointments are usually every four weeks during this one-year breast course.

Olaparib tablets 300 mg orally twice daily continuously, with source renal/toxicity/interaction adjustments. Tablets are not interchangeable with capsules. Breast duration:one year or earlier recurrence/unacceptable toxicity.

Checks and follow-up

FBC, renal and liver profile every 4 weeks throughout the 12-month breast course; earlier/additional tests for symptoms or abnormal results. Review adherence, tablet supply, nausea, fatigue and all medicines. No routine olaparib-specific ECG, echo or DXA; follow separate previous/ongoing treatment risks. Breast surveillance continues after olaparib, without automatic asymptomatic CT or tumour-marker scheduling.

The whole-treatment map shows the checks for this exact treatment setting.

Support between reviews

Nausea support matched to oral treatment

NCCP classifies oral olaparib as moderate-to-high emetic risk. Agree an anti-sickness plan under local policy, and seek review if oral intake falls. The SmPC describes early/intermittent symptoms; do not automatically copy an intravenous chemotherapy antiemetic cycle.

Check the antiemetic interaction

Aprepitant is a moderate CYP3A inhibitor in NCCP 00588 and can increase olaparib exposure. The prescriber/pharmacist should choose and reconcile supportive medicines; never add an antiemetic from another regimen without checking.

Blood and infection support

Anaemia or other low counts can lead to interruption, dose adjustment, transfusion or additional investigation. There is no universal routine G-CSF course for every person on this regimen.

Medicines and supplements

Review all prescriptions, over-the-counter drugs and herbal products. Strong/moderate CYP3A inhibitors or inducers can change exposure. Avoid grapefruit products and seek advice before starting anything new.

Stomach symptoms

NCCP allows consideration of a proton pump inhibitor under local policy. This is symptom- and patient-dependent, rather than a compulsory medication for everyone.

Review the full medicine plan

NCCP classifies oral olaparib as moderate-to-high emetic risk. Agree an anti-sickness plan under local policy, and seek review if oral intake falls. The SmPC describes early/intermittent symptoms; do not automatically copy an intravenous chemotherapy antiemetic cycle. Aprepitant is a moderate CYP3A inhibitor in NCCP 00588 and can increase olaparib exposure. The prescriber/pharmacist should choose and reconcile supportive medicines; never add an antiemetic from another regimen without checking. Anaemia or other low counts can lead to interruption, dose adjustment, transfusion or additional investigation. There is no universal routine G-CSF course for every person on this regimen. Review all prescriptions, over-the-counter drugs and herbal products. Strong/moderate CYP3A inhibitors or inducers can change exposure. Avoid grapefruit products and seek advice before starting anything new. NCCP allows consideration of a proton pump inhibitor under local policy. This is symptom- and patient-dependent, rather than a compulsory medication for everyone. Agree an oral antiemetic prevention / rescue plan before treatment. Continuous oral treatment does not use the Day 1–4 IV chemotherapy template. Confirm agent, dose, frequency, duration and interactions; no default daily steroid order.

Oncology daytime: Insert numbers here. **Out of hours:** Insert numbers here. Follow your own hospital’s urgent-care plan. Emergency: 112 / 999.



Your complete treatment reference

https://sactcheck.com/patient/00588/scope.html

Public information only. No personal information is encoded.